PDA describes a profile where everyday demands feel genuinely threatening — including demands the person actually wants to meet. It's the one on this shelf where even the name is under active debate.

In plain words

PDA usually stands for Pathological Demand Avoidance, though many people in the community prefer Persistent or Pervasive Drive for Autonomy, arguing that 'pathological' stigmatises. Others keep the original, saying it honestly conveys how far outside conscious control it sits. Both views come from people living it. It is not a standalone diagnosis in the DSM; in the UK it's sometimes noted as a demand-avoidant profile within autism.

What it can feel like from the inside

- A request landing like a threat, even a kind one, even one you agree with.
- Wanting to do the thing and being unable to the moment it becomes expected.
- Praise making it worse, because now it's expected again next time.
- Negotiating, deflecting, or going quiet — anything to keep some control.
- For some, none of this shows outwardly at all; it turns inward instead.

What people often get wrong

THE ASSUMPTION	WHAT'S ACTUALLY GOING ON
"They're just defiant."	Defiance is a choice. This is a nervous system reading demands as danger.
"Be firmer with them."	Escalating demands escalates the threat. It reliably makes things worse.
"They manage at school, so it's parenting."	Holding it together all day is why it collapses at home, where it's safe.
"They can't be, they're so easy-going."	Internalised PDA is quieter and often missed entirely.
"It's not a real thing."	It isn't a formal DSM diagnosis. It's still a useful description of a real pattern.

What often comes with it

NOT COMPENSATION PRIZES — GENUINE, USEFUL THINGS

Strong sense of justice and personal autonomy.

Creative, imaginative, often socially perceptive.

Excellent negotiators, from constant practice.

Thrive genuinely when given real choice and control.

Question rules that deserve questioning.

What actually helps

- ☐ Offer choices, not instructions
- ☐ Reduce demands before adding strategies
- ☐ Drop praise that raises expectation
- ☐ Phrase things indirectly ('I wonder if…')
- ☐ Give control over timing and order
- ☐ Treat collapse as distress, not defiance

If someone tells you this is them

HELPFUL	LESS HELPFUL
“Whenever you're ready.”	“You need to do this now.”
“Would you rather A or B?”	“Because I said so.”
“No pressure — genuinely.”	“See, you managed it!”

Autonomy isn't a luxury here. It's the thing that makes everything else possible.

A friendly explainer written for the community — not a diagnostic tool and not medical advice. If you want an assessment, your GP is the place to start. Nobody experiences this identically, so take what fits and leave the rest.

Free to share — print it, forward it, hand it to anyone who needs it. More free guides, one per neurotype, at froglogic.co.uk